

China Healthcare: ASCO 2026 Preview for China Pharma/Biotech: Takeaways from abstracts

Ahead of the 2026 American Society of Clinical Oncology annual meeting (ASCO 2026, May 29–June 2 in Chicago), regular abstracts have been released. In our [previous preview](#), we put this year's focus of China datapoints on readouts that can potentially disrupt front-line treatment standards of NSCLC. Since the lifting of the abstract embargo, we have already seen sac-TMT (TROP2 ADC, Kelun Biotech) showing [bar-setting survival results](#) from the ph3 OptiTROP-Lung05 trial, while waiting for the announcement of overall survival (OS) data of ivonescimab (PD-1/VEGF, Akeso) from the ph3 HARMONi-6 trial on May 31. Beyond pivotal ph3 readouts, newly released and updated data from earlier-stage trials indicate an evolving landscape on a variety of indications; here we highlight: 1) Advancements in 1L NSCLC for potential disruption (**Innovent**, **3SBio**, **CStone**); 2) evolving SCLC landscape, including long-tail survival updates from alvelumab (DLL3/DLL3/CD3 TstCE, Zelgen) and new data from ivonescimab and in 2L+ setting; 3) reaffirmed survival benefit of IBI363 (PD-1/IL-2 α -bias BsAb, **Innovent**) in late-line squamous NSCLC; 4) positive ph1 readouts from **BeOne**'s solid tumor pipelines; 5) mCRPC landscape being shaped by B7-H3 therapies and PROTAC (**Hengrui**); and 6) emerging CLDN18.2 therapies in early-line gastric cancer (GC) (**SBP**).

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1L AGA-NSCLC: Continued shaping of new treatments

Besides the highly anticipated ivonescimab and sac-TMT readouts, 1L NSCLC has seen several other noteworthy updates that warrant closer inspection as the reshaping of standard for 1L treatment is taking place (see [Exhibit 1](#)). We highlight:

- 1. IBI363 (PD-1/IL-2 α -bias BsAb, Innovent)** reaching a cORR of 81.8% in combo with chemo (vs. ivonescimab: 75.9% from HARMONi-6, sac-TMT: 70.2% from OptiTROP-Lung05, pembro: 48.3% from KEYNOTE-189) at a special dosing scheme of 3-1.5mg/kg (starting with 3mg/kg plus chemo in cycle 1, then lowered to 1.5mg/kg Q3W in subsequent cycles), which not only delivered higher ORR than single-level doses (1.5mg/kg: 42.1% cORR, 3mg/kg: 57.1%), but also came with better tolerability with a relatively lower G3+ TEAEs rate of 65.2% (vs. 1.5mg/kg: 82.1%, 3mg/kg: 93.1%). While the “induction-maintenance” dosing strategy was designed to obtain a more balanced therapeutic index, we expect more discussions on the underlying mechanisms given that this dosing scheme showed better ORR than the 3mg/kg cohort and better safety than the

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1.5mg/kg cohort.

- BioNTech** announced preliminary data for pumitamid (PM80002, PD-L1/VEGF-A BsAb) + chemo from a Ph2/3 trial (ROSETTA Lung-02). Pumitamid showed a comparable 70% ORR (vs. ivonescimab: 75.9%, sac-TMT: 70.2%), but a milder G3+ TRAE at 44.2% (vs. ivonescimab 63.9%, sac-TMT 55.3%). PFS and OS are not released in the abstract, which require continuous monitoring for read-across.
- 3SBio** updated results for **SSGJ707 (PD-1/VEGF)** as monotherapy after extended follow-up (15.2m), with a cORR of 67.6% (vs. previous ORR of 61.8%, 10 mg/kg, N=34; vs. ivonecimab: 50%, pumitamid: 47.1%) and mPFS of 12.4m (vs. ivonecimab: 11.1m, pumitamid: 10.9m), exhibiting competitive efficacy. More details on the safety profile were released in the abstract, with a relatively higher rate of G3+ TRAE of 42.2% (vs. previous 24.1%, vs. ivonescimab: 29.4%, pumitamid: 18%), including VEGF-related G3+ AE rate at 18.1% (vs. ivonescimab 10.2%).
- Recall **CStone** shared preliminary data of **CS2009 (PD-1/VEGF/CTLA-4 TsAb)** in 1L PD-L1 positive (TPS≥50%) NSCLC in March, suggesting potent tumor response with ORR of 90% from a small sample size (N=10). The company will disclose updated efficacy and safety data in c.50 patients at the conference, and we see the updated results as important PoC for this “next-gen” PD-1/VEGF TsAb in 1L NSCLC.

Exhibit 1: Potential disruptors in the ramp-up competition in 1L NSCLC

Treatment landscape of 1L NSCLC

Molecule	SKB264 (TROP2 ADC)	Ivonescimab (PD-1/VEGF)		SSGJ707 (PD-1/VEGF)		Pumitamid (PD-L1/VEGF)		IBI363 (PD-1/IL-2α-bias)
Developer	Kelun Biotech / Merck	Akeso / Summit		3S Bio / Pfizer		BioNTech / Biotheus		Innovent
Study	OptiROP-Lung05	HARMONI-6	AK112-303 (HARMONI-2)	28-Nov-25	8-Jan-25	ROSETTA Lung-02	PM8002-A001 Cohort 6	
Data cutoff date	29-Sep-2025	28-Feb-25	29-Jan-24	28-Nov-25	8-Jan-25	21-Nov-25	20-Dec-23	22-Dec-25
Follow-up period, median	10.5 months			15.2 months				5.8 months
NCT no.	NCT06448312 (phase 3)	NCT05840016 (phase 3)	NCT05499390 (phase 3)	NCT06361927 (phase 2)	NCT06412471 (phase 2)	NCT06712316 (phase 2)	NCT05918445 (phase 1b/2a)	NCT06468098 (phase 1)
Dosage	sac-TMT (4 mg/kg IV Q2W) + pembro (400 mg IV Q6W)	ivo + chemo vs. Tisle + chemo	mono (ivo: 20mg/kg vs Keytruda: 200mg Q3W)	mono (5-30mg/kg, Q3W)	+ chemo	+ chemo	mono	+ chemo
Treatment line	1L PD-L1 TPS ≥1%	1L	1L	1L	1L	1L	1L	1L
Baseline								
N	208 (sac-TMT+P) vs. 205 (P)	266 vs 266	398	83	108	44	61	80
Prior treatment	None	None	None	None	None	None	None	None
Median age (yrs)	65	≥65 (49.2% vs 47.7%)	≥65 (54.3%), <65 (45.7%)			66		64
ECOG PS=1	85%	84.2% vs 83.5%	87.20%			61.4%		81.0%
Efficacy								
ORR	70.2% vs. 42%	75.9% vs 66.5% PD-L1 TPS: < 1% (69.5% vs 61.0%); ≥ 1% (80.1% vs 70.2%)	50.0% vs 38.5%	cORR 67.6% (N=34, 10mg/kg), nsq: 63.3% (12/22), sq: 75% (9/12), PD-L1 TPS: 1-49% (61.9%); ≥ 50% (76.9%)	nsq: 58.3% sq: 81.3% (10mg/kg)	70% (28/40) nsq: 66.7% (1400mg: 72.7%; 2000mg: 60%); N=21 sq: 73.7% (1400mg: 81.8%; 2000mg: 62.5%) PD-L1 TPS: <1% (20/35); 1-49% (9/35); ≥50 (6/35)	47.1%	3-1.5mg/kg: 86.4% (nsq: 87.5%; sq: 85.7%) cORR 81.8% , N=22 1.5mg/kg: 57.9%, 42.1% , N=19 3mg/kg: 66.7%, 57.1% , N=21 PD-L1 TPS: <1% (65.2%); 1-49% (34.8%)
DCR		90.6% vs 89.1%	89.9% vs 70.5%			100%		100%
DOR, m		11.20 vs 8.38	NR	nsq: NR, sq: 6.2				NR
PFS, m	NR vs. 5.7 (HR=0.35, 0.26-0.47, p<0.0001) PD-L1 TPS: 1-49% HR=0.28 (0.19-0.41) ≥ 50% HR=0.47 (0.29-0.77)	11.14 vs 6.90 (HR=0.60) PD-L1 TPS: < 1%: HR=0.55 ≥ 1%: HR=0.66 1-49% HR=0.63 ≥ 50%: HR=0.71	11.14 vs 5.82 (HR=0.51) sq: HR=0.50 nsq: HR=0.55	12.4 nsq: 12.4, sq: 8.9, PD-L1 TPS: 1-49% (9.6); ≥ 50% (12.4)		10.9		
OS, m	HR=0.55 (0.36-0.85)		HR=0.777 (interim at 39% maturity)					
AE								
G3+ AE	TEAE: 55.3% vs. 31.4%	63.9% vs 54.3%	29.4% vs 15.6%	42.2%	8.9% (10mg/kg)	44.2%	18% (11/61)	81.3% (65.2% for 3-1.5mg/kg; 82.1% for 1.5mg/kg; 93.1% for 3mg/kg)
SAE		32.3% vs 30.2%	20.8% vs 16.1%	0			9.8% (6/61)	
TR death		3.0% vs 3.8%	0.5% vs 1.0%					
AE of concern	G3+ TEAE: neutrophil count decreased (17.3%), anemia (9.1%), stomatitis (5.3%)	G3+ VEGF related AE: 7.5% vs 2.3%; G3+ hemorrhagic events occurred in 1.9% vs 0.8%, for ivonescimab and tislelizumab groups	G3+ VEGF related AE: 10.2% vs 1.0%; G3+ proteinuria: 3.1% vs 0%; G3+ haemorrhage: 1.0% vs 0.5%; G3+ arterial thromboembolism: 1.0% vs 0	G3+ VEGF related AE: 18.1%; irAEs: 8.4%; G3+ TRAEs: pneumonia (8.4%), hypertension (7.2%), and hemoptysis (6.0%)		irAEs: 14% (6/43) irAEs: 37.7% (23/61) All grade TRAEs (≥15%): hypertension, hypothyroidism, proteinuria, hypoalbuminemia, hypocalcemia, aspartate aminotransferase increase, and lactate dehydrogenase increase		G3+ TEAE: anemia (18.8%), neutrophil count decrease (42.5%), white blood cell count decrease (20.0%), arthralgia (2.5%), and platelet count decrease (17.5%)
Discontinue	5.3% vs. 4.9%	1.1% vs 2.3%		3%		5%	8.2% (5/61)	

Newly announced data at ASCO 2026 is highlighted on its data cutoff date

Source: ASCO 2026, ESMO 2025, ASCO 2025, ASCO 2024, WCLC 2024

SCLC: ZG006 reiterated the long-tail survival of DLL3 TCE in late-line setting

SCLC has seen new updates, led by Zelgen’s ph1/2 update of alveltamig (ZG006, DLL3/DLL3/CD3 TstCE) with highlight on the OS benefit in 3L+ setting: 12m OS rate of 65.9% / 59.2% for 10mg / 30mg, implying potential 15-20 months of median OS (vs. 13.6m for tarlatamab in 2L). In 2L+ setting, alveltamig’s DOR and PFS also appeared to be differentiated compared to other competing treatments, with mDOR of 12.6 months (ZL-1310: 6.1m; tarlatamb: 6.9m), 12-month PFS rate of 50.5% (ZL-1310: mPFS 5.4m; tarlatamb: mPFS 4.2m) (see [Exhibit 2](#)). Preliminary data from abstract looks highly encouraging, and we will look into the patient base data of alveltamig’s ph1/2 to better understand the asset’s positioning amid competition. Aside from DLL3-targeting therapies, Ph1 data of Akeso/SMMT’s ivonescimab in combo with liposomal irinotecan in 2L SCLC showed a 62% ORR and 9.8-month mPFS, and **SKB500 (B7H3 ADC, Kelun Biotech)** showed 71% ORR, reinforcing our view that cORR of 60+% is the bar for 2L+ SCLC.

Exhibit 2: Zelgen’s alveltamig can potentially bring superior survival benefit in 2L+ SCLC, and ivonescimab released data in 2L SCLC

Treatment landscape of 2L+ SCLC

Molecule	Ivonescimab	SKB500	ZL-1310	IDE849 (SHR-4849)	Tarlatamab	Alveltamig (ZG006)	Alveltamig (ZG006)	LB2102
Developer	Akeso	B7-H3 ADC Kelun Biotech	Zai Lab/MedLink	IDEA/VAHengru	Amgen	Zelgen	Zelgen	Legend
MoA	PD-1/VEGF BsAb	218mg/kg Q3W + liposomal irinotecan	DLL3 ADC	DLL3 ADC	DLL3xCD3 TCE	CD3xDLL3xDLL3 TCE	CD3xDLL3xDLL3 TCE	DLL3 CAR-T
Dosage			0.6/1.6/2.4/2.8 mg/kg Q3W	2.4/3.0/3.5/4.2 mg/kg Q3W	10mg Q2W	10mg, 30mg Q2W	0.1/0.3/1/3/10/30/60mg Q2W	0.3-16.0x10E6 CAR+T cells/kg
Study			Part 2 dose-expansion AACR-NCI-EORTC '25	Dose-expansion WCLC 2025	Del.Lph-304 (ph3) ASCO 2025	ASCO 2026	ASCO 2026	ASCO 2026
Data presented at	ASCO 2026	ASCO 2026	15-Sep-2025	20-Jun-2025	28-Jan-2025	30-Sep-2025	ESMO 2024	WCLC 2025
Data cutoff date	15-Dec-2025	23-Dec-2025	15-Sep-2025	20-Jun-2025	11.2mos (tarlatamab) / 11.7 mos (chemo)	13.5 months	16 months	5-Jan-2026
Follow-up months	7.3 months	7.1 months	7.1 months	3.5 months	11.2mos (tarlatamab) / 11.7 mos (chemo)	c.3 months	16 months	120.5 days
NCT no. (stage)	NCT06478043 (phase 2)	NCT06736327 2L+ SCLC	NCT06179069 (phase 1a)	NCT06443489 (phase 1)	NCT05740566 2L+ SCLC	NCT06283719 (phase 2)	NCT05979284 (phase 1/2)	NCT05680922 (phase 1)
Treatment line	2L SCLC	2L+ SCLC	2L+ SCLC	2L+ SCLC	2L+ SCLC	3L+ SCLC	2L+ SCLC	2L+ SCLC
Baseline	N	60	102	100 (71 evaluable SCLC)	254 / 255	60	31 (4/15/12 on 10/30/60 mg)	17
Prior treatment		21	43.5% ≥2 prior lines Prior DLL3 therapy: 9.6%	2 (1-4)	Post 1 / line of PBC	24 (10mg) / 24 (30mg)	24 (21 SCLC)	12
Brain metastases	35%	34.5% ≥2 prior lines	32%	33%/15% after 2/3+ prior lines	44% vs 45%	100%/100% ≥2 lines 45.8%/37.5% ≥3 lines	71% ≥2 prior lines	median 2 (1-5)
Median age (y)	62		65	59.5	64 vs 66	29.2% / 29.2%	4%	65%
ECOG PS=1	88%		70%	98.1%	67% vs 69%	91.7% / 91.7%		56.5
Efficacy								
ORR	cORR 61.7%	71.4% (15/21)	68% for 2L SCLC (13/19 at 1.6mg/kg) All 2L: 80% All 2L+ 50% brain meta: 66%	cORR: 47.9% (34/71) 60.9% for 2L SCLC 70% (7/10, 2L SCLC at 2.4mg/kg) brain meta: 83.3% (5/6) at 2.4mg/kg	no brain meta: 54.5%, 61.9% brain meta cORR: 50%, 44.4% no liver meta: 50%, 64.7% liver meta: 58.3%, 46.2% previous Checkpoint	cORR 74.2%	33.3% (7/21) 10mg or higher dose: 66.7% (6/9)	18% (3/17)
DCR	91.7%	100%	94% for 2L SCLC (18/19 at 1.6mg/kg) All 2L: 90% All 2L+ 88% brain meta: 94%	83.0% 77.1% for 2L SCLC 80% (8/10, 2L SCLC at 2.4mg/kg)	70.8% (10mg) 66.7% (30mg)	88.9% in pts treated at 10mg or higher doses	76%	70%
DOR, m			6.1	6.7 (NR for 2L)	6.9 vs 5.5	12.6 (12m-rate: 65.3%)	7 (208 days)	
PFS, m	9.8 CFI >=90 days: 11.9 CFI <90 days: 7 NR		5.4	6.7 (NR for 2L)	4.2 vs 3.7 (HR=0.71)	12m-rate: 50.5%		3.48
OS, m					13.6 vs 8.3 (HR=0.60)	NR, 12m-rate: 65.9%, 59.2%	12m-rate: 74.2% 18m-rate: 58.6%	
AE								
G3+ AE	26.7%	16.5%	TRAEs: 20% (n=100 SCLC and NEC)	TRAEs: 48% (n=100 SCLC and NEC)	TRAEs: 27% / 62%	TEAEs: 29.2% / 45.8% TRAEs: 20.8% / 37.5%	17%	TRAEs: 23.5% (4/17)
SAE			8%	16%	28% vs 31%	20.8% vs 33.3%		8.3%
TR death	0%	0%	nil at 1.2 or 1.6 mg/kg	0	0.4% vs 2%	0 / 4.2% (1/24)	0	G3+ TRAEs:
AE of concern	G3+ TRAEs: decreased neutrophil count (8.3%), decreased white blood cell count (8.3%), fatigue (6.7%), and diarrhea (3.3%)	G3+ TRAEs were anemia, lymphocyte count decreased, pneumonia, and asthenia (each 3.1%)	G3+ TRAEs: anemia (9.6%), neutropenia (11.3%)	Most common G3+ TRAEs: decreased WBC (27%), neutropenia (33%), anemia (6%)	CRS: 56% vs 1% (primarily G1/2) G3+ AEs: anemia (4% vs 29%), neutropenia (6% vs 23%), pneumonia (6% vs 8%)	All grade CRS: 41.7% / 75%, G3+ CRS: 0 / 8.3% ICANS: 0 / 8.3%	Mainly G1/2 TRAEs, 46% CRS, one G3 CRS (4%); No DLTs and ICANS	G3+ TRAEs: anemia (16.7%), WBC decreased (16.7%) hypoxia (all n = 1 at DLT)
ILD			Two G1(2.8%, 2/72)					
Discontinuation	0%	0%	nil at 1.2 or 1.6 mg/kg	2%	3% vs 6%	0 / 0	0	
Dose reductions	25%		nil at 1.2 / 1.6 mg/kg	15%		12.5% / 20.2%		

mCRP, metastatic castration-resistant prostate cancer; SCLC, small cell lung cancer; ESCC, esophageal squamous cell carcinoma; sqNSCLC, squamous non-small cell lung cancer; DOR, duration of response; NE, not estimable; NR, not reported; ORR, objective response rate; OS, overall survival; PFS, progression-free survival

Newly announced data at ASCO 2026 is highlighted on its data cutoff date

Source: ASCO 2026, ASCO 2025, AACR-NCI-EORTC '25, WCLC 2025

Late-line NSCLC: Matured survival data brings more visibility on IBI363 in late-line sq-NSCLC

With the latest Ph1 data update from **IBI363** (PD-1/IL-2α-bias BsAb), 3mg /kg dose exhibits improved overall survival benefit in IO-treated squamous NSCLC cohort (see [Exhibit 3](#)), for which the data showed mPFS of 10.1m (vs. 9.3m in previous readout) and 18.2 months mOS, reaffirming our view on its differentiation (vs. mPFS 2.8m/mOS 7.6m for Dato-DXd, mPFS 3.9m /mOS 9.4m for docetaxel in 2L+ sq-NSCLC). mOS for the

non-squamous subset was shorter than sq-NSCLC (15.2m vs. 18.2m), which appeared more consistent (vs. [ASCO2025 data](#)) with the PFS observation (4.2m, vs. 10.1m) between the two groups. The safety profile also updated as the rate of G3+ AE adjusted to 48.5% (from 43.9%). We believe the updated results will consolidate IBI363's focus on squamous subset and facilitate its regulatory progress in PBC/IO-treated sq-NSCLC. For late-line NSCLC, 1) **Henlius** also updated its pooled analysis of **HLX43 (PD-L1 ADC)** from Ph1 and Ph2 studies with encouraging ORR trend, reporting 47.4% ORR in EGFRwt non-sq NSCLC and 36.4% ORR in sqNSCLC. (vs. previous 33.3%); 2) **SBP** released preliminary data of its EGFR/c-Met bispecific ADC **TQB6411**, with ORR of 50% from a small cohort of 10 patients (vs. 40% ORR for amivantimab, EGFR/c-Met BsAb, J&J).

Exhibit 3: More visibility on IBI363 in late-line sq-NSCLC

Updates of IBI363 and HLX43 in 2L+ NSCLC

Molecule		HLX43				IBI363 (PD-1/L2 ^{non-sq} BsAb)							
Developer		Henlius				Innovent							
Study		HLX43-FIH101/HLX43-NSCLC201		HLX43-FIH101/HLX43-NSCLC201		CIBI363A102				CIBI363A102			
Data cutoff date		31-Dec-2025		22-Oct-2025		20-Nov-2025				7-Apr-2025			
Follow-up period, median		~4.0 months		~4.0 months		14.4 months				OS: 17.3 months OS: 11.3 months OS: 17.7 months OS: 10.2 months			
NCT no.		NCT06115642/NCT06907615 (Ph1/Ph2 pooled analysis)		NCT06115642/NCT06907615 (pooled analysis)		NCT05460767 (phase 1a/1b)				NCT05460767 (phase 1a/1b)			
Regimen (Dosage)		Mono		Mono		Mono				Mono			
Treatment line		2L+ nsq 2L+ sq		3L+ EGFRwt nsq 3L+ EGFRwt sq		2L+				2L+			
Baseline	N	85 pts at 2.5mg/kg Q3W	89 pts at 2.0mg/kg Q3W	85 pts at 2.5mg/kg Q3W	89 pts at 2.0mg/kg Q3W	28 sq pts at 1/1.5mg/kg	31 sq pts at 3mg/kg	30 non-sq pts without AGA at 0.6/1/1.5mg/kg	25 non-sq pts without AGA at 3.0mg/kg	28 sq pts at 1/1.5mg/kg	31 sq pts at 3mg/kg	30 non-sq pts without AGA at 0.6/1/1.5mg/kg	25 non-sq pts without AGA at 3.0mg/kg
	Prior treatment	median 2 (1-9)		median 2 (1-7)		IO+PBC treated ≥2 prior lines: 64.3%	IO+PBC treated ≥2 prior lines: 67.7%	IO+PBC treated ≥2 prior lines: 80%	IO+PBC treated ≥2 prior lines: 64%	IO+PBC treated ≥2 prior lines: 64.3%	IO+PBC treated ≥2 prior lines: 67.7%	IO+PBC treated ≥2 prior lines: 80%	IO+PBC treated ≥2 prior lines: 64%
	Median age (yrs)	62		59		58	61	61.5	63	58	61	61.5	63
	ECOG	PS 1: 71.8%		PS 1: 77.5%		PS 1: 67.9%	PS 1: 83.9%	PS 1: 76.7%	PS 1: 80.0%	PS 1: 67.9%	PS 1: 83.9%	PS 1: 76.7%	PS 1: 80.0%
Efficacy	ORR	47.4% (9/19) in EGFRwt (50% in EGFRmut)	36.4% (12/33) 40% (DTX failed, 3L+, 6/15)	48.6% (17/35)	38.5% (11/33) (DTX failed, 3L+)	25.9% (7/27) cORR: 25.9%	43.3% (13/30) cORR: 36.7%	17.2% (5/29) cORR: 13.8%	26.0% (7/25) cORR: 24.0%	25.9% (7/27) cORR: 25.9%	43.3% (13/30) cORR: 36.7%	17.2% (5/29) cORR: 13.8%	26.0% (7/25) cORR: 24.0%
	DCR			94.3%	75.8% (84.6% (DTX failed, 3L+))	66.7%	90.0%	62.1%	76.0%	66.7%	90.0%	62.1%	76.0%
	DOR, m					10.2	NR	21.3	NR	10.2	NR	NR	NR
	PFS, m					5.5	10.1	2.7	4.2	5.5	9.3	2.7	5.6
AE	OS, m					12.5	18.2	17.5	15.2	15.3	12m rate: 70.9%	17.5	12m rate: 71.6%
	G3+ AE	42.9%		28.2%	24.7%	TEAEs: 48.5%				TRAEs: 17.7% (1/1.5mg/kg), 43.9% (3mg/kg)			
	SAE					21.0% (1/1.5mg/kg), 40.4% (3mg/kg)				21.0% (1/1.5mg/kg), 40.4% (3mg/kg)			
	TR death				One case of respiratory failure			0.7%				0%	
AE of concern		G3+ TEAEs: anemia (n = 25, 12.2%), lymphocyte count decreased (n = 47, 22.9%), white blood cell count decreased (n = 27, 13.2%), and neutrophil count decreased (n = 23, 11.2%)		G3+ TEAEs: anemia (11.8%), lymphocyte count decreased (10.6%), neutropenia (11.8%), thrombocytopenia (1.7%)		G3+ TRAEs: arthralgia (3.7%), anemia (4.4%), and rash (8.8%)				G3+ TRAEs at 3mg/kg were mainly rash and arthralgia			
Discontinue		8.3%		3.5%	2.2%	8.1%				6.5% (1/1.5mg/kg), 7.0% (3mg/kg)			

Newly announced data at ASCO 2026 is highlighted on its data cutoff date

Source: ASCO 2026, ASCO 2025

Positive ph1 data from solid tumor pipelines of BeOne

BeOne Medicines released Ph1 data for several solid tumor assets with competitive efficacy/safety profile to peers:

- CDK4 inhibitor (BGB-43395)** showed a largely on-par response rate as Pfizer's CDK4 inhibitor atirmociclib in a similar 1L setting (combo with letrozole) in HR+/HER2- breast cancer at much shorter follow-up, with ORR of 58% / 68% at 240mg/400mg dose (with 6.8m of follow-up, vs. 60.6%/69.7% cORR for atirmociclib with 16.5m/24.8m of follow-up, see [Exhibit 4](#)), while we expect to see more matured data at the presentation, where ORR could further deepen with longer follow-up. The safety profile remained largely consistent with previous data, with higher rates

of GI AEs (79-95% / 5-30% of All Grade / G3+ diarrhea, 26-60% of All Grade vomiting for 240-600mg dose levels) and lower rates of blood AEs (e.g. c.20% or lower for All Grade neutropenia, anemia with rare G3+ cases, vs. 62%/27% of All Grade/G3+ neutropenia for atirromociclib). In particular, we highlight that 240mg showed the best therapeutic profile, while 600mg appeared less tolerable (e.g. 30% of G3+ diarrhea).

2. **GPC3/4-1BB bispecific antibody (BGB-B2033)** showed a noticeable 28.9% cORR in 2L+ hepatocellular carcinoma (HCC) with only 8.2% patients experiencing G3+ TRAE, which may indicate significant improvement from current standard systemic therapy in 2L+ HCC (vs. cabozantinib, ORR 4%, G3+ TRAE 68%, see [Exhibit 5](#)). Compared to other 4-1BB BsAbs, we note that the safety profile of BGB-B2033 appeared to be more benign with lower G3+ TRAE (8% vs. 21%/19% for LBL-024/QLF31907), which in our view leaves room for further exploration. Readacross to other newly presented data from the GPC3 landscape (also on ASCO26), MRG006A (Lepu, GPC3 ADC) had ORR of 23.1% and G3+ TRAE of 42.3% in 2L+ HCC, whereas Ori-C101 (Oricell, GPC3 CAR-T) showed potent cORR of 66.7% at RP2D in 3L+ HCC, though we will focus more on duration of response for autologous CART, as initial response to cell therapies likely appears high.
3. Preliminary results from B7-H4 ADC (BG-C9074) in a bucket of solid tumors with an overall cORR of 28.1%. It had 31.3% ORR in 2L+ triple-negative breast cancer (TNBC), comparable to Hansoh's B7-H4 ADC (HS-20089, ORR 33.3%), and 34.5% ORR in 2L+ ovarian cancer compared to AZ's newly announced 24.4% ORR for puxi-sam (AZD8205) from a Ph1/2a study.

Exhibit 4: BGB-43395 showed largely on-par response rate as atimociclib in 1L HR+/HER2-BC

Comparison between BGB-43395 and atimociclib plus letrozole in HR+ breast cancer

Molecule	BGB-43395		Atimociclib (PF-07220060)		
Company	BeiGene		Pfizer		
MOA	CDK4		CDK4		
NCT No.	NCT06120283		NCT04557449		
Trial design	Part C	Safety expansion	Part 2B		
Phase	Ph1		Ph1/2a		
Regimen	+letrozole	+letrozole	+ letrozole	+ letrozole	+ET (letrozole or fulvestrant!)
Data cutoff date	23-Sep-24	11-Nov-25	21-Feb-25	1-Nov-23	1-Nov-23
Median follow-up (months)	1.8	6.8	24.8	16.5	
Dosing	DL 2-8	240mg / 400mg / 600mg	300mg BID	300mg BID	300mg/400mg BID
Baseline					
Size	15	58 (19 / 19 / 20)	34	34	34
Median age (years)	54		59	59	59
ECOG PS of 1	46.7%		55.9%	55.9%	55.9%
Prior regimens	post CDK 4/6i ET and chemo	CDK4/6i-naïve	CDK4/6i-naïve	CDK4/6i-naïve	CDK4/6i-naïve
Median prior regimens	6 (1-9)	0	0	0	0
Efficacy data					
ORR	58% / 68% / 40%		69.7%	60.6%	
mPFS (months)	NR				
Adverse events (AE)					
G3+ TEAEs	3.7% (1/15)	32% / 37% / 65%			nil
G3+ TRAEs	nil				
Serious AEs	6.7% (1/15)			5.9%	
Dose reduction					5.9%
Discontinuation	6.7% (1/15)		2.9%	2.9%	5.9%
TR Death					
AE of interest (All/G3+)	TRAEs	TEAEs	TRAEs	TRAEs	TRAEs
Diarrhea	60% (nil)	79% / 95% / 90% (5% / 11% / 30%)			
Decreased appetite	6.7% (nil)				
Fatigue	13.3% (nil)				
Nausea	33.3% (nil)	53% / 68% / 85% (0% / 5% / 0%)			
Vomiting	6.7% (nil)	26% / 47% / 60% (0% / 0% / 0%)			
Neutropenia	6.7% (nil)	26% / 21% / 20% (0% / 0% / 10%)	61.8% (26.5%)	61.8% (23.5%)	61.8% (23.5%)
Leukopenia			41.2% (nil)		38.2% (nil)
Thrombocytopenia		5% / 0% / 5%			
Anemia		5% / 21% / 20% (0% / 0% / 5%)			23.5% (nil)

Newly announced data at ASCO 2026 is highlighted on its data cutoff date

Source: ASCO 2026, Miami Breast Cancer Conference 2026, Miami Breast Cancer Conference 2025

Exhibit 5: Emerging GPC3-targeting therapies exhibiting potent response in 2L+ HCC

Treatment landscape of 2L+ HCC

SoC

		2L+ HCC			
Molecule		BGB-B2033	MRG006A	Ori-C101	Irpagratinib (ABSK-011)
Developer		BeOne	Lepu	Oricell	Abbisko
Combo		Mono	Mono		Mono
MoA		GPC3/4-1BB BsAb	GPC3 ADC	GPC3 CAR-T	FGFR4i
Study				BEACON	ABSK-011-101
Data cutoff date		10-Dec-2025	19-Dec-2025	24-Dec-2025	5-Sep-2024
Follow-up period, median		3.9 months	5.7 months		5.2 months
NCT no.		NCT06427941 (phase 1)	NCT07093970 (phase 1/2)	NCT05652920 (phase 1b)	NCT04906434 (phase 1)
Dosage			1.6-6.4 mg/kg Q3W		160/220/300 mg BID
Treatment line		2L+ HCC	2L+ HCC	3L+ HCC	2L+ FGF19+ HCC
Baseline	N	61 (60 with HCC)	26	19	74 (n=22/40/12)
	Prior treatment	≥1 systemic therapy	median 2 (1-4)	median 3 (2-8)	≥2 prior line: 64.9% prior ICI: 85.1% prior ICI+mTKI: 75.7%
	Median age (yrs)				52.0
	HBV				
	BCLC stage B/C				89.2% BCLC C
	ECOG PS=1				66.2% PS 1
Efficacy	ORR	cORR 20.3% (12/59) cORR 28.9% (11/38) at doses above prelim response dose	23%	cORR 50% (9/18) cORR 66.7% at RP2D	36.8% (14/38 at 220mg BID) 44.8% (13/29 at 220mg BID pICI+TKI)
	DCR		68%	77.8% (14/18)	78.9% (30/38 at 220mg BID) 79.3% (23/29 at 220mg BID pICI+TKI)
	DOR, m		4.2		7.4
	PFS, m		7.0		5.5 (220mg BID) 5.5 (220mg BID pICI+TKI)
	OS, m			14.4	5.2 vs 1.9 (HR=0.44)
AE	G3+ AE	8.2%	42.3%	G3+ CRS: 42.1%, 0 ICANS	28.4% (40% at 220mg BID)
	SAE				18%
	TR death				0
	AE of concern	G3+ TRAEs: ALT increased, aspartate aminotransferase increased, blood bilirubin increased, drug eruption, lymphopenia and neutrophil count decreased (1.6% each)			G3+ TRAEs (n=40 at 220mg BID): AST increased (12.5%), ALT increased (15.0%), diarrhea (7.5%)
	Discontinue				4.1%

Note: BCLC: Barcelona Clinic liver cancer stage; ALT, Alanine transaminase; AST, Aspartate transaminase.

Newly announced data at ASCO 2026 is highlighted on its data cutoff date

Source: ASCO 2026, NEJM, ESMO 2024

Prostate cancer: B7-H3 ADC / AR PROTAC continued shaping up mCRPC landscape

With DB-1311 (B7H3 ADC, Duality/BioNTech) showing promising efficacy in CRPC late-line treatment (PFS of 11.3m post Lu-177), we see more readouts from new competitors in this indication including: 1) **HRS-5041 (Hengrui)** as the potential first AR PROTAC to enter phase 3, achieved PFS of 11.0m in pretreated mCRPC with oral dose; 2) YL201 (B7H3 ADC, Medilink) showed PFS of 9m under similar setting; 3) HS-20093 (B7H3 ADC, Hansoh/GSK) had 9m-PFS rate of 55.7%. (see [Exhibit 6](#)). Beyond prostate cancer and SCLC, B7-H3 ADC showed potential efficacy across broader types of solid tumor, such as **SKB500 (Kelun Biotech)** with ORR of 55.6% in ESCC and **SYS6043 (CSPC)** with 38.5% ORR in cervical cancer.

Exhibit 6: B7H3 ADC / AR PROTAC are reshaping the mCRPC treatment landscape

Treatment landscape of B7H3 ADC / AR PROTAC mCRPC

Molecule	DS-7300 (I-DXd)	DB-1311/BNT324	HS-20093	YL201	HRS-5041
Developer	Daiichi Sankyo/MSD	Duality/BioNTech	Hansoh/GSK	MediLink	Hengrui
MoA	B7H3 ADC	B7H3 ADC	B7H3 ADC	B7H3 ADC	AR PROTAC
Dosage	4.8-16.0 mg/kg	6mg/kg; 9mg/kg Q3W		2mg/kg, 2.4mg/kg	30–540mg QD or 240–300mg BID
Study	DS7300-A-J101				
Data presented at	ESMO 2023	ASCO GU 2026	ASCO GU 2026	ASCO 2026	ASCO 2026
Data cutoff date	31-Jan-2023	5-Sep-2025		12-Dec-2025	12-Dec-2025
Follow-up	16.6 months	9.2 months	8.7 months	12.2 months	
NCT no. (stage)	NCT04145622 (phase 1/2)	NCT05914116 (phase 1/2)	NCT06001255 (phase 2)	NCT06241846 (phase 2)	NCT05942001 (phase 1)
Treatment line	2L+	2L+	2L+	2L+	2L+
Baseline	N				
	59	104	50	82	156
Prior treatment	Median=6 (1-11)	Median=4 (1-14)	Median=2 (1-8)	Median=4 (1-8)	
Metastases		Bone (87.7%) Lymph node (47.9%) Liver (23.3%) Lung (21.9%)	100% Hormonal, 72% Taxane	100% has ARAR, 70.7% chemo	
Median age (y)	68	70	68	61	
ECOG PS=1	PS=1 (73.3%)	71%	72%	66%	
ORR	25% (15/59)	41.4% (24/58) cORR: 34.5% (30% post Lu-177)	33.3% (11/33 pt with target lesion)	29.5% (33.3%, 24%)	240mg BID: 42.9% (AR-LBD mutation), 11.1% (AR-LBD WT)
PSA50 ORR		35.4% (30.4% post Lu-177)	32% (16/50)	38.5% (48.9%, 24.2%)	240mg BID: 48.4% (AR-LBD mutation), 20.0% (AR-LBD WT)
DCR		87.9%			
DOR, m	6.4	10.2		9.9 (11.8, 9.0)	
PSA DOR, m		8.4			
PFS, m	5.3	11.3 (11.3 post Lu-177), 6m 72%, 9m 63%	NE, 9m-rate: 55.7%	rPFS, m:9.1	240mg BID: 11.0 (AR-LBD mutation), NR (AR-LBD WT)
OS, m	13.0	6m 91.7%, 9m 88.2%			
G3+ AE	47%	42.5% (31/73)	54.0%	43.9%	12.8%
SAE			20%		2.6%
TR death		0			
AE of concern	G3+ TEAEs (≥3% of pts): anemia (19%), neutropenia (4%), nausea (3.4%), and lymphocyte count decreased (3.4%)	Most common TRAEs: nausea and hematological events, primarily G1–2; Hematologic TRAEs occurred more frequently with 9mg/kg		G3+ TRAEs: neutropenia (29.3%), anemia (22.0%), leukopenia (18.3%), and thrombocytopenia (9.8%)	
ILD	one G4 in 12mg/kg, and one G5 in 16mg/kg	Four G1		0%	
Discontinuation	4%	10%		1.2%	
Dose reductions		16%			

Newly announced data at ASCO 2026 is highlighted

Source: ASCO 2026, ASCO GU 2026, ESMO 2023

Gastric cancer: Emerging new CLDN18.2-targeting options for first-line setting

For HER2- GC front-line treatment, we see emerging data from CLDN18.2 ADCs with potential to beat current SoC zolbetuximab, showing better efficacy and tolerable safety profile:

- LM-302 (CLDN18.2 ADC, SBP/LaNova)** in combination with toripalimab/chemo reached ORR of 69.2% (vs 53.8% ORR for zolbe+chemo) and PFS of 12.55m (vs 6.8m for zolbe+chemo, see [Exhibit 7](#)), though addition of PD-1 may contribute to the

better efficacy.

2. QLS31905 (CLDN18.2/CD3 TCE, Qilu Pharma) plus chemotherapy showed broad efficacy for patients with both high and low CLDN18.2 expression - ORR of 74%/77.8% for overall/CLDN-18.2 low GC patients. For GC late-line treatment, data from CLDN18.2/4-1BB BsAb looks less encouraging though preliminary, such as SHR-3821 (Hengrui), which showed confirmed ORR of 11.1%/0% in 15/20mg.

Besides CLDN18.2 regimens, we note the latest FGFR2 data in GC, **ABSK061 (small-molecule FGFR2/3 inhibitors, Abbisko)** in combo with oral PD-L1 with/without CAPOX, showed encouraging preliminary therapeutic profile with competitive ORR (90% in 1L with triplet regimen, N=10; 31% in 2L+ without CAPOX, N=16), and tolerable G3+ TEAE (51%, mainly from blood AEs related to chemo) with note on the well-controlled ocular AE (one G3 case, or 3%, vs. 28% for bemarituzumab/mFOLFOX6 in ph2 FIGHT trial) which is the focus for FGFR2-targeting regimens that could negatively impact patient compliance.

Exhibit 7: New treatments are emerging in 1L GC

Treatment landscape of 1L HER2- GC

		1L HER2- GC		1L HER2-/CLDN18.2+ GC/GEJ		
		SoC	FGFR2/3i	SoC	ADC	BsAb
Molecule		Nivolumab (PD-1)	ABSK061	zolbetuximab	LM-302	QLS31905 (CLDN18.2/CD3)
Developer		BMS	Abbisko	Astellas Pharma	LaNova / SBP	Qilu Pharma
Study		CheckMate-649		GLOW		
Follow-up, median		12.1 months		12.6 months	16.16/14.82 months	
NCT no.		NCT02872116 (phase 3)	NCT06632262 (phase 2)	NCT03653507 (phase 3)	NCT05188664 (phase 2)	NCT06041035 (phase 1b/2)
Combo		+ CAPOX/FOLFOX	+ PD-L1 CAPOX	+ CAPOX	+toripalimab chemo	+chemo
Dosage		360mg Q3W	75mg	600mg/m ² Q3W	1.8mg/kg Q2W	500/800 µg/kg Q3W
Treatment line		1L HER2- LA mGC/GEJ (PD-L1 CPS ≥5)		1L CLDN18.2+ LA mGC/GEJ (CLDN18.2 2+/3+ IHC ≥ 75%)	1L CLDN18.2+/HER2- GC/GEJ	1L CLDN18.2+ GC/GEJ
Baseline	N	789 (nivo + chemo) vs 792 (CAPOX or FOLFOX)	37 (13 1L, 24 2L+)	254 (zolbetuximab + CAPOX) vs 253 (PBO + CAPOX)	71 (39 +T / 32 +T+chemo)	43 (9 low CLDN18.2)
	Prior treatment	0	0	0	0	0
	Median age (y)					
	ECOG					
Efficacy	ORR	60% vs 45%	90% (9/10)	53.8% vs 48.8%	64.5% / 69.2% (CLDN18.2 ≥25%) 14.3% / 40% (CLDN18.2 <25%)	74% (77.8% in low CLDN18.2)
	DCR				96.8% / 96.2% (CLDN18.2 ≥25%) 71.4% / 100% (CLDN18.2 <25%)	93% (100% in low CLDN18.2)
	PFS, m	7.7 vs 6.0 (HR=0.68)		8.21 vs 6.80 (HR=0.69, p=0.0007)	10.68 / 12.55 12.22 / NE (CLDN18.2 ≥25%) 5.72 / 6.8 (CLDN18.2 <25%)	10.09 (NE, 9m rate 83.33% in low CLDN18.2)
	OS, m	14.4 vs 11.1 (HR=0.71)		14.39 vs 12.16 (HR=0.77, p=0.0118)	NE (CLDN18.2 ≥25%) 10.78 / NE (CLDN18.2 <25%)	
AE	G3+ AE	59% vs 44%	51.4%	72.8% vs 69.9%	66.7% / 81.3%	60.5% (26/88)
	SAE	22% vs 12%		47.2% vs 49.8%		
	TR death	2% vs <1%		2.4% vs 2.8%	0	0
	AE of concern		G3+ TEAE: platelet count decreased (23.1%), hypokalaemia (15.4%) and neutrophil count decreased (15.4%) FGFR-related Aes: ocular events (2.7% G3), stomatitis (2.7% G3)	All grades: nausea (68.5% vs 50.2%), vomiting (66.1% vs 30.9%), and decreased appetite (41.3% vs 33.7%)	G3+: neutrophil count decreased and WBC decreased	
	Discontinuation	36% vs 24%			15.4% / 31.3%	9%

Newly announced data at ASCO 2026 is highlighted on its molecule name

Source: ASCO 2026, ASCO Plenary Series 2023, Lancet

Disclosure Appendix

Reg AC

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